Appendix 3 (Page 1 of 3):

Department of Haematology/Oncology (Schiehallion)
Royal Hospital for Children
1345 Govan Road
Glasgow G51 4TF
Tel: 0141 452 4475 / 4476

Information for Patients being treated with Targeted Therapy

Date: _____

PATIENT'S NAME, DOB & CHI
ADDRESS
POSTCODE

To Whom It May Concern

The above named patient is receiving targeted therapy, but can continue on the national childhood immunisation programme with the exception of live vaccines (MMR, Rotavirus).

Yours faithfully

PRINT NAME: _____

Contact details Tel. & E-mail _____

NMC/ GMC number: _____

cc Parents of (add Patient Name)
cc GP
cc (GGC Patients) Susan Hurley

Paediatric Haematology/Oncology Unit
Royal Hospital for Children, Glasgow

NHS Greater Glasgow and Clyde

Appendix 3 (page 2 of 3):

Department of Haematology/Oncology (Schiehallion)
Royal Hospital for Children
1345 Govan Road
Glasgow G51 4TF
Tel: 0141 452 4475 / 4476

Information for Patients being treated with Targeted Therapy who require booster immunisations

Date: _____

PATIENT'S NAME, DOB & CHI
ADDRESS
POSTCODE

To Whom It May Concern

The above named patient is receiving targeted therapy and THEY SHOUD NOT RECEIVE ANY LIVE VACCINES, but as they have also had treatment with cytotoxic chemotherapy (completed more than 3 months ago) they should receive booster immunisations for the following diseases:

Pathogen protected against	**Vaccine**	**Trade Name** (Equivalent alternative may be used)
Seasonal Influenza	SIIV	Various
SARS-CoV-2**	SARS-CoV-2 vaccine (per national guidance)	Various
Diphtheria, tetanus, pertussis, polio, *Haemophilus influenzae* b, Hepatitis B	DTaP/IPV/Hib/HepB	Infanrix hexa or Vaxelis
Meningococcal B	Men B	Bexsero
Streptococcus pneumoniae	PCV20 (or PCV13)	Prevenar20 (or Prevenar13)
Meningococcal ACWY	Men ACWY-conjugate	Nimenrix or Menveo
Human Papillomavirus (see point 2 below)	HPV-9*	Gardasil 9

Vaccines Names : DTaP = Diphtheria/ Tetanus/ acellular Pertussis, Hib = H. influenzae b conjugate, HepB =Hepatitis B, HPV-9 = 9-valent Human papillomavirus vaccine, IPV = Inactivated polio virus, Men B = Meningococcal B, Men ACWY = Meningococcal ACWY conjugate, PCV20 = 20-valent Pneumococcal conjugate, PCV13 = 13-valent Pneumococcal conjugate, SIIV = Seasonal inactivated influenza vaccine

Yours faithfully

PRINT NAME: _____

Contact details Tel. & E-mail _____

NMC/ GMC number: _____

cc *Parents of (add Patient Name)*
cc *GP*
cc *(GGC Patients) Susan Hurley*

Appendix 3 (page 3 of 3):

Further Information / Guidance

If primary vaccination schedule was not completed pre-treatment the patient should start afresh and receive the complete primary schedule aside from live vaccines.

* HPV-9 vaccine should be offered to girls and boys ≥12 years old: For HPV vaccine naïve, two doses of HPV vaccine should be given at 0 and 4-6 months from starting vaccination. For girls and boys that completed the course prior to diagnosis, a booster dose should be given.

If the named vaccine is not available, an alternative with the same composition may be used.

**SARS-CoV-2 vaccine may be given as per national guidance. For more information, please look at the full vaccination guideline on the CCLG website - https://www.cclg.org.uk

Multiple inactivated vaccines can be co-administered preferably at different sites (in a different limb), however, they may be given over several appointments as per Green Book guidance on spacing intervals (at the discretion of the local immunisation team).

Give booster immunisations no sooner than 3 months after completion of chemotherapy treatment.

Prevenar® (1 dose if child previously vaccinated with **Prevenar®** **OR** 2 doses - 2 months apart if no previous vaccination of **Prevenar®**)

Some of the above vaccines can be administered at the same visit at different sites and the child recalled as soon as possible for the remaining vaccines e.g. the following week or month.

Subsequent routine booster doses (e.g. pre-school) will **not** be necessary if the child is scheduled for these within the next 12 months.

When the child is due a school leaving booster of Revaxis® ensure there is a 5 year interval between receiving the above Infanrix hexa® and the school leaving booster.

Inactivated influenza – every autumn for as long as the child is considered at risk from influenza virus infection. The intranasal live-attenuated influenza vaccine should not be used in patients or their household contacts. Note that the immune response to influenza vaccine is not optimal during the first 6 months after therapy, which is the period of greatest risk; therefore inactivated influenza vaccination should be offered to family members/household contacts.